

Research Question: How do developed countries utilize their financial stability to positively impact lower socioeconomic levels, particularly in areas like education and healthcare, and why is this approach often lacking in Pakistan?

1. How Developed Countries (UK and UAE) Utilize Financial Stability

Developed countries like the UK and UAE harness their robust economic resources—characterized by high GDP per capita, strong tax revenues, and stable fiscal systems—to fund equitable education and healthcare systems, directly benefiting lower socioeconomic groups. Below, I outline the mechanisms and outcomes in each sector, supported by data from the provided datasets and additional context.

Education

- **United Kingdom:**
 - **Financial Commitment:** The UK allocates approximately 5.5% of GDP to education (SE.XPD.TOTL.GD.ZS, 2021), equating to £49.8 billion annually (2020/21, UK government data). This high investment ensures free primary and secondary education, with near-universal adjusted net enrollment rates (99.9% in 2021, SE.PRM.TENR).
 - **Impact on Lower Socioeconomic Groups:**
 - **Access and Equity:** Free education and targeted programs (e.g., Pupil Premium, £2.6 billion in 2021) support disadvantaged students, reducing dropout rates and improving outcomes. For instance, 74% of disadvantaged pupils achieve expected standards in reading and math by age 11 (2022).
 - **Skill Development:** Vocational training and apprenticeships (e.g., 5.3 million apprenticeships since 2010) enhance employability, lifting low-income groups out of poverty. The Gini index (SI.POV.GINI, 34.2 in 2021) reflects moderate inequality, partly mitigated by education-driven mobility.
 - **Child Labour Prevention:** High enrollment and strict labour laws eliminate child labour, ensuring children from low-income families remain in school.
 - **Mechanism:** High tax-to-GDP ratio (33.5% in 2021, OECD) and efficient public spending enable sustained investment, reducing socioeconomic disparities.
- **United Arab Emirates:**
 - **Financial Commitment:** The UAE allocates 3.9% of GDP to education (2021), with AED 10.4 billion (14.8% of the 2020 federal budget) dedicated to

education. Oil revenues and a diversified economy (GDP per capita \$44,300, NY.GDP.PCAP.CD, 2021) fund these expenditures.

- **Impact on Lower Socioeconomic Groups:**
 - **Universal Access:** Primary enrollment is 99.8% (2021), with free public schools for citizens and subsidized education for expatriates. Scholarships (e.g., Sheikh Hamdan Program) support low-income students.
 - **Workforce Development:** Technical and vocational education (e.g., ACTVET programs) equips low-skilled workers, including expatriates, for high-demand sectors, reducing unemployment (3.1%, SL.UEM.TOTL.ZS, 2021).
 - **Poverty Reduction:** Education contributes to low poverty rates (<2% at \$2.15/day, estimated), as skilled workers access better jobs.
- **Mechanism:** Centralized budget allocations and public-private partnerships (e.g., Emirates Foundation) ensure resources reach underserved communities, particularly expatriate workers.

Healthcare

- **United Kingdom:**
 - **Financial Commitment:** Health expenditure is 11.9% of GDP (SH.XPD.CHEX.GD.ZS, 2021), or £277 billion for the NHS (2021/22). High GDP per capita (\$49,700) and tax revenues fund universal healthcare.
 - **Impact on Lower Socioeconomic Groups:**
 - **Equitable Access:** The NHS provides free healthcare at the point of use, ensuring low-income groups access services without financial burden. Life expectancy is 80.7 years (SP.DYN.LE00.IN, 2022).
 - **Poverty Mitigation:** Free healthcare prevents medical impoverishment, with only 0.7% of households facing catastrophic health costs (WHO, 2020).
 - **Health Outcomes:** Targeted programs (e.g., Healthy Start vouchers) improve nutrition and health among low-income families, reducing health disparities.
 - **Mechanism:** Progressive taxation and a centralized NHS model prioritize equity, with social protection systems (e.g., disability benefits) complementing health services.
- **United Arab Emirates:**

- **Financial Commitment:** Health expenditure is 5.7% of GDP (2021), supported by oil wealth and mandatory insurance schemes (e.g., Taa-meen, Thiqa). The 2021 health budget was AED 58.2 billion.
- **Impact on Lower Socioeconomic Groups:**
 - **Access:** Citizens receive free healthcare, while expatriates (70% of population) benefit from employer-sponsored insurance. Life expectancy is 78.4 years (2022).
 - **Preventive Care:** Mobile clinics and free screenings (e.g., Dubai Health Authority initiatives) target low-income workers, reducing disease burden.
 - **Poverty Reduction:** Low out-of-pocket costs (18% of health spending, WHO) prevent financial distress among low-wage workers.
- **Mechanism:** Strategic investments in healthcare infrastructure and insurance mandates ensure broad coverage, supported by fiscal surpluses.

Social Protection Synergies

- **UK:** Robust social protection (e.g., Universal Credit, £69 billion in 2021) supports low-income households, reinforcing education and healthcare access. Low unemployment (4.2%, SL.UEM.TOTL.ZS, 2021) reflects education-driven employability.
- **UAE:** Labour protections (e.g., MoHRE services) and wage support for expatriates reduce poverty, complementing education and health investments. High labour force participation (87.2%, SL.TLF.ACTI.ZS, 2021) reflects economic inclusion.

2. Why This Approach Is Lacking in Pakistan

Pakistan's inability to replicate the UK and UAE's approach stems from a combination of financial constraints, structural inefficiencies, and socio-cultural barriers. Below, I detail these challenges, supported by data and interrelations from the analysis.

Financial Constraints

- **Low Economic Capacity:**
 - Pakistan's GDP per capita is \$1,505 (NY.GDP.PCAP.CD, 2021), compared to \$44,300 (UAE) and \$49,700 (UK). This limits fiscal resources for social spending.
 - The tax-to-GDP ratio is 10.2% (2021, World Bank), among the lowest globally, constraining public budgets. Tax evasion and a narrow tax base exacerbate this issue.
 - High public debt (87% of GDP, 2021) and fiscal deficits (7.1% of GDP) divert funds to debt servicing (40% of budget) rather than social sectors.

- **Impact:**
 - **Education:** Spending is 2.8% of GDP (2021), below the global average of 4%. This results in 23 million out-of-school children (40% of school-age population, UNESCO) and low primary enrollment (78.5%, SE.PRM.TENR, 2021).
 - **Healthcare:** Health expenditure is 2.9% of GDP (2021), leading to high out-of-pocket costs (60% of health spending, WHO). Life expectancy is 66.1 years (2022), reflecting poor access and quality.
 - **Comparison:** The UK's 11.9% and UAE's 5.7% health spending dwarf Pakistan's, enabling universal or near-universal coverage absent in Pakistan.

Structural Inefficiencies

- **Weak Institutions:**
 - Corruption (Pakistan ranks 140/180 on Transparency International's 2021 Corruption Perceptions Index) diverts resources from social programs. For example, education funds are often mismanaged, reducing school infrastructure quality.
 - Inefficient budget execution (e.g., only 60% of allocated education funds utilized in Punjab, 2020) limits impact.
- **Fragmented Social Protection:**
 - Programs like the Benazir Income Support Program (BISP) cover only 7.2% of the population (2021), reaching 26% of the poor. Lack of coordination between federal and provincial programs reduces effectiveness.
 - Comparison: The UK's Universal Credit and UAE's MoHRE services are centralized, ensuring broader coverage and efficiency.
- **Impact:**
 - **Education:** Low spending and inefficiencies result in poor school quality, driving child labour (83% linked to poverty and lack of schooling, ILO). Low enrollment perpetuates low labour force participation (58.3%, SL.TLF.ACTI.ZS, 2021).
 - **Healthcare:** Fragmented health systems and low investment lead to 44.7% of Multidimensional Poverty Index (MPI) deprivation tied to health access (UNDP, 2021).

Socio-Cultural Barriers

- **Poverty and Child Labour:**

- Poverty headcount at \$2.15/day is 27.6% (SI.POV.DDAY, 2020), compared to <2% in UAE and UK. Poverty forces children into labour (3.3 million child labourers, ILO), reducing school attendance.
- Cultural norms prioritizing early marriage or work for boys in rural areas (e.g., 60% of rural children out of school) exacerbate educational gaps.
- **Gender Disparities:**
 - Female account ownership is 16.3% (FX.OWN.TOTL.FE.ZS, 2021) vs. 44.0% for males, limiting women's economic participation. This contrasts with UAE (86.7% female) and UK (99.9% female).
 - Gender gaps in education (female enrollment 72% vs. male 85%, 2021) restrict female labour force participation (22%, SL.TLF.ACTI.ZS), perpetuating poverty.
- **Impact:**
 - **Education:** Socio-cultural barriers and poverty drive low enrollment, increasing reliance on inadequate social protection and sustaining poverty cycles.
 - **Healthcare:** Limited financial inclusion (32.3% account ownership, FX.OWN.TOTL.ZS, 2021) and gender disparities restrict access to health services, increasing out-of-pocket costs and health-related poverty.

Policy and Implementation Gaps

- **Historical Failures:**
 - Initiatives like the Social Action Programme (SAP, 1990s) failed due to poor implementation and lack of sustainability, unlike the UK's sustained NHS funding or UAE's Vision 2021 health goals.
 - Recent programs (e.g., Ehsaas) show promise but lack scale, covering only 10% of the poor (2021).
- **Economic Vulnerability:**
 - Reliance on agriculture (22% of GDP, 40% of employment) and informal sectors (80% of workforce) limits economic growth and tax revenues.
 - Climate shocks (e.g., 2022 floods costing \$30 billion) divert funds from social sectors, unlike the UAE's climate-resilient economy or UK's diversified tax base.
- **Impact:**
 - **Education and Healthcare:** Limited fiscal space and policy failures result in underfunded systems, unable to address socioeconomic disparities effectively.

- **Comparison:** The UK and UAE's proactive policies (e.g., UAE's mandatory health insurance, UK's education reforms) contrast with Pakistan's reactive, underfunded approaches.

3. Interrelations and Data Insights

The analysis highlights how finance, education, healthcare, poverty, and social protection interact, amplifying disparities in Pakistan compared to the UK and UAE:

- **Finance and Healthcare:** Pakistan's low GDP per capita and financial inclusion (32.3%) limit health budgets, leading to high out-of-pocket costs and low life expectancy. The UK's high financial stability funds the NHS, while UAE's insurance mandates ensure access.
- **Education and Child Labour:** Pakistan's low enrollment (78.5%) and spending drive child labour, unlike the UK and UAE's universal education systems, which eliminate it.
- **Poverty and Social Protection:** Pakistan's high poverty (27.6%) and weak social protection strain education and health systems. The UK's benefits and UAE's labour protections reduce poverty's impact.
- **Interesting Fact:** Pakistan's female account ownership (16.3% in 2021) is lower than the UAE's total ownership in 2011 (59.7%), underscoring a financial inclusion gap that limits health and education investments for women.

4. Recommendations for Pakistan

To emulate the UK and UAE's approach, Pakistan must address its financial and structural challenges:

- **Increase Fiscal Space:**
 - Raise the tax-to-GDP ratio to 15% by 2030 through progressive taxation and reduced evasion, potentially adding \$10 billion annually (World Bank).
 - Restructure debt to free up 20% of budget for social spending.
- **Enhance Education:**
 - Increase education spending to 4% of GDP, targeting rural and female enrollment to reduce child labour by 50% (ILO estimate).
 - Expand conditional cash transfers (e.g., BISP's education stipends) to boost enrollment by 10 million children.
- **Strengthen Healthcare:**
 - Raise health spending to 5% of GDP, focusing on primary care and reducing out-of-pocket costs to 30%.

- Implement universal health coverage pilots, learning from UAE's insurance models.
- **Unify Social Protection:**
 - Integrate BISP and Ehsaas into a single platform, covering 50% of the poor by 2030.
 - Adopt UK-style unemployment benefits to support 2 million unemployed workers.
- **Address Socio-Cultural Barriers:**
 - Launch awareness campaigns to promote female education and financial inclusion, targeting a 50% increase in female account ownership by 2030.
 - Enforce child labour laws, supported by education subsidies.
- **Learn from UK/UAE:**
 - Adopt UAE's targeted budget allocations for education and health, tailored to Pakistan's context.
 - Emulate the UK's centralized, equitable service delivery for scalability.

5. Conclusion

The UK and UAE utilize financial stability—through high GDP, robust tax systems, and strategic allocations—to fund education and healthcare, significantly benefiting lower socioeconomic groups. Universal education and healthcare systems, supported by strong social protection, reduce poverty, child labour, and inequality, fostering inclusive growth. Pakistan's approach is hindered by low financial resources, structural inefficiencies, fragmented policies, and socio-cultural barriers. High poverty, low tax revenues, and inadequate social spending perpetuate disparities, limiting education and healthcare access. By increasing fiscal space, unifying social programs, and addressing cultural gaps, Pakistan can begin to mirror the successes of developed countries, breaking poverty cycles and enhancing human development.